NCCP 00105 · VERSION 7

Your treatment, explained

Advanced melanoma · NCCP 00105 v7 · Document revised 6 November 2026

> **Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Ipilimumab alone — advanced melanoma

Reference treatment cycle

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp

Reference contents

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

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UNDERSTAND YOUR TREATMENT

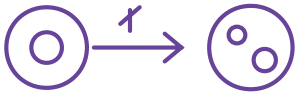
What each medicine does

To help immune cells control advanced melanoma using ipilimumab alone; the planned infusion course is limited, but immune effects and follow-up continue beyond it.

CTLA-4 CHECKPOINT IMMUNOTHERAPY

Ipilimumab

Yervoy



Releases a CTLA-4 immune brake

Ipilimumab blocks CTLA-4, one of the signals that normally restrain T cells. This may help immune cells attack melanoma. The same immune activity can inflame healthy organs, sometimes after the last infusion; some hormone problems need lasting treatment.

Releases a CTLA-4 immune brake → Treatment plan and safety checks → Report new symptoms early

What to watch for

Report diarrhoea, blood in stools, severe headache, unusual weakness or spreading rash promptly.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Head / hormones

Immunotherapy

Severe headache, dizziness, profound tiredness, thirst or altered vision can signal pituitary, adrenal, thyroid or glucose problems.

3 Heart

Immunotherapy

Rare heart inflammation can cause chest pain, palpitations, faintness or breathlessness. Severe chest pain, collapse or major breathing difficulty

5 Liver

Immunotherapy

Liver inflammation may first appear in blood tests. Yellow eyes or skin, dark urine, severe nausea or right-sided abdominal pain also need prompt

7 Nerves / muscles

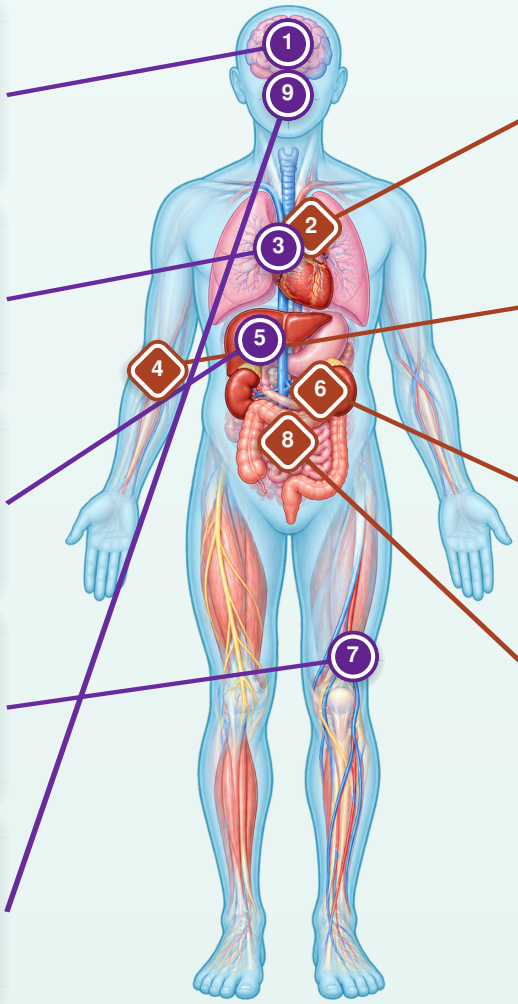
Immunotherapy

New weakness, drooping eyelids, double vision, numbness or difficulty walking, swallowing or breathing may be a rare nerve or muscle effect.

9 Eyes

Immunotherapy

Eye pain, redness, light sensitivity or changed vision can indicate inflammation. Get prompt assessment; do not assume it is ordinary tiredness.



2 Lungs

Immunotherapy

New cough, shortness of breath or chest discomfort can mean lung inflammation or infection. Report changes promptly, even with normal

4 Skin

Immunotherapy

Rash and itching can occur. Rapid spreading, blistering, peeling or sores involving the mouth, eyes or genitals require urgent assessment.

6 Kidneys / urine

Immunotherapy

Kidney inflammation may be detected in blood tests. Report reduced urine, blood in urine, swelling or a marked change in thirst.

8 Bowels

Immunotherapy

New diarrhoea, blood or mucus, severe cramps or abdominal pain can mean bowel inflammation. Record changes from your usual pattern and call early; severe pain or bleeding needs urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Your treatment phases

21-DAY REFERENCE CYCLE

Treatment visits

Treatment visits **Repeat only as prescribed**

Ipilimumab by IV infusion once every 21 days, up to four doses. Complete within 16 weeks of the first dose, whichever limit comes first.

Ipilimumab by IV infusion once every 21 days, up to four doses. Complete within 16 weeks of the first dose, whichever limit comes first.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00105 v7: current regimen source

THE WHOLE TREATMENT JOURNEY

Ipilimumab alone — advanced melanoma: treatment and checks

Advanced melanoma

Ipilimumab by IV infusion once every 21 days, up to four doses. Complete within 16 weeks of the first dose, whichever limit comes first.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00105	Blood count, kidney and liver function, glucose and thyroid tests. Review autoimmune disease, previous immune reactions, transplantation, infection, pregnancy possibility and all medicines, including steroids.	Blood count, kidney/ liver tests, glucose and thyroid tests before every cycle, plus a careful symptom review. Cortisol and other tests are added when symptoms suggest endocrine or other immune effects. Normal routine blood results do not exclude colitis, lung, nerve or other inflammation.	Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.	Immune toxicity can start or persist after the four-dose course. Keep the alert card, follow-up appointments, steroid-taper or hormone-replacement plan and the individual scan schedule. Report new symptoms after treatment has ended.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Scans and tumour results NCCP 00105	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00105	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1

Treatment visits

21-day reference cycle

To help immune cells control advanced melanoma using ipilimumab alone; the planned infusion course is limited, but immune effects and follow-up continue beyond it.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00105

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Ipilimumab by IV infusion once every 21 days, up to four doses. Complete within 16 weeks of the first dose, whichever limit comes first.

Treatment timing: displayed days 1–21

Treatment or check	Days in this window	What happens
Treatment visit	Day 1	The source dose is ipilimumab 3 mg/kg over 90 minutes on day 1 of each 21-day cycle, with observation for one hour after the infusion. Allow additional time for symptom review, blood results and preparation. Report any previous infusion reaction before treatment.
Between-visit monitoring	Days 2–21	Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.

Schedule and review: Ipilimumab by IV infusion once every 21 days, up to four doses. Complete within 16 weeks of the first dose, whichever limit comes first.

Holds, missed treatment and phase changes: If a visit is missed or held, contact the unit. Do not count a cancelled visit as a dose received. The team reviews whether another dose remains appropriate within the four-dose and 16-week limits; do not assume a delayed dose can always be made up.

Why these bands stay broad: This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00105 v7: current regimen source ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Complete timing and symptom notes

Day 1: Treatment day

The source dose is ipilimumab 3 mg/kg over 90 minutes on day 1 of each 21-day cycle, with observation for one hour after the infusion. Allow additional time for symptom review, blood results and preparation. Report any previous infusion reaction before treatment. Report symptoms and previous reactions before treatment.

Day 2–21: Between treatment visits

Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy. Do not wait for the next appointment to report urgent symptoms.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Taking or receiving treatment

The source dose is ipilimumab 3 mg/kg over 90 minutes on day 1 of each 21-day cycle, with observation for one hour after the infusion. Allow additional time for symptom review, blood results and preparation. Report any previous infusion reaction before treatment.



STEP 2

Missed, vomited or held treatment

If a visit is missed or held, contact the unit. Do not count a cancelled visit as a dose received. The team reviews whether another dose remains appropriate within the four-dose and 16-week limits; do not assume a delayed dose can always be made up.



STEP 3

Appointments and supply

Arrange blood tests, transport, medicine supply and interpreter or carer support. Check who reviews results and how you receive the decision before the next dose.

STEP 4

Paper copies and contacts

Keep the full handbook, three-page summary and wallet card together. The QR code is optional. Ask the team to replace every labelled example contact with your real oncology and out-of-hours details; ask for a new printed plan when treatment changes.

Sources: NCCP 00105 v7: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Supportive medicines and symptom plan

NCCP classifies ipilimumab as low emetogenic risk; anti-sickness treatment is individual. Routine infusion premedication is not usually required, but previous reactions may change the plan. Do not start steroids or alter prescribed steroid tapers or hormone replacement without the team.

2 Skin and daily care

Use gentle skin care and sun protection. Report rash and itching early. Spreading rash, skin pain, blisters, peeling or sores affecting the mouth, eyes or genitals need urgent assessment. Continue melanoma skin self-checks and planned skin reviews.

3 Fertility, pregnancy and breastfeeding

Discuss pregnancy prevention, fertility and breastfeeding before treatment. Effective contraception and the duration after the last dose must be confirmed against the current ipilimumab product information and your written plan; this guide does not borrow the nivolumab five-month interval. Tell the team promptly if pregnancy is possible.

Sources: nccp

TREATMENT REFERENCE · 01

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Your diagnosis and stage

Keep the pathology report, original melanoma site, stage and known sites of spread. This guide concerns advanced disease that cannot be removed completely by surgery or has spread; it does not describe a postoperative recurrence-prevention course.

Sources: nccn-patient

The tumour test

A BRAF V600 test helps identify whether BRAF-targeted medicines are an option. A tumour mutation does not by itself mean an inherited family condition. Keep the exact result and ask how it changes your choices.

Sources: nccn-patient

Scans and local treatments

Record the scans used to follow known disease. Surgery or radiotherapy may have a separate role for a particular site; ask for their timing and risks to be added to the overall plan.

Sources: nccn-patient

New changes still matter

Report a new lump, changing mole, persistent skin sore or unexplained symptom. A response at one site does not establish that every other symptom is harmless.

Sources: nccn-patient

TREATMENT REFERENCE · 02

Your medicines and overall plan

To help immune cells control advanced melanoma using ipilimumab alone; the planned infusion course is limited, but immune effects and follow-up continue beyond it.

The schedule

Ipilimumab by IV infusion once every 21 days, up to four doses. Complete within 16 weeks of the first dose, whichever limit comes first.

Sources: nccp

How treatment works

Ipilimumab blocks CTLA-4, one of the signals that normally restrain T cells. This may help immune cells attack melanoma. The same immune activity can inflame healthy organs, sometimes after the last infusion; some hormone problems need lasting treatment.

Sources: nccp

Important boundaries

This guide describes ipilimumab monotherapy at 3 mg/kg. It is not the nivolumab combination and does not automatically lead to nivolumab maintenance. A four-dose treatment course does not mean immune risks end after dose four.

Sources: nccp

Evidence and choices

Median overall survival for ipilimumab alone was 10.1 versus 6.4 months with gp100; HR 0.66, p=0.003. These are group results from that study, not a prediction for you. Historical gp100 comparator and selected previously treated population. Do not substitute the separate ipilimumab-plus-gp100 arm or compare indirectly with modern PD-1 treatment.

Sources: hodi2010 · nccp

TREATMENT REFERENCE · 03

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Administration

The source dose is ipilimumab 3 mg/kg over 90 minutes on day 1 of each 21-day cycle, with observation for one hour after the infusion. Allow additional time for symptom review, blood results and preparation. Report any previous infusion reaction before treatment.

Sources: nccp

Missed or held treatment

If a visit is missed or held, contact the unit. Do not count a cancelled visit as a dose received. The team reviews whether another dose remains appropriate within the four-dose and 16-week limits; do not assume a delayed dose can always be made up.

Sources: nccp

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Sources: nccp

Practical arrangements

Know where and when bloods are taken, how results reach the prescriber, and how to obtain the next supply or appointment. Ask for written instructions in a format you can use without a phone.

Sources: nccn-patient

TREATMENT REFERENCE · 04

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Blood count, kidney and liver function, glucose and thyroid tests. Review autoimmune disease, previous immune reactions, transplantation, infection, pregnancy possibility and all medicines, including steroids.

Sources: nccp

During treatment

Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, plus a careful symptom review. Cortisol and other tests are added when symptoms suggest endocrine or other immune effects. Normal routine blood results do not exclude colitis, lung, nerve or other inflammation.

Sources: nccp

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

Sources: nccp

Response and follow-up

Immune toxicity can start or persist after the four-dose course. Keep the alert card, follow-up appointments, steroid-taper or hormone-replacement plan and the individual scan schedule. Report new symptoms after treatment has ended.

Sources: nccp

TREATMENT REFERENCE · 05

What to expect between visits

A quiet treatment day does not rule out later problems.

Act on new symptoms

Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.

Sources: nccp

Supportive medicines

NCCP classifies ipilimumab as low emetogenic risk; anti-sickness treatment is individual. Routine infusion premedication is not usually required, but previous reactions may change the plan. Do not start steroids or alter prescribed steroid tapers or hormone replacement without the team.

Sources: nccp

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

Sources: nccp

When life is affected

Tell the team about fatigue, pain, appetite, sleep, mood and ability to work or care for others. Practical and supportive care can accompany anticancer treatment; you do not have to wait until treatment ends.

Sources: nccn-patient

TREATMENT REFERENCE · 06

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

Skin and sun

Use gentle skin care and sun protection. Report rash and itching early. Spreading rash, skin pain, blisters, peeling or sores affecting the mouth, eyes or genitals need urgent assessment. Continue melanoma skin self-checks and planned skin reviews.

Sources: nccp

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Sources: nccp

Food, fluids and activity

Keep meals and fluids manageable and follow any individual fluid or diet restrictions. Gentle activity and rest can be adjusted to how you feel. Ask for dietetic or rehabilitation help if intake, weight, strength or mobility is falling.

Sources: nccn-patient

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Sources: nccp

TREATMENT REFERENCE · 07

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Contraception and pregnancy

Discuss pregnancy prevention, fertility and breastfeeding before treatment. Effective contraception and the duration after the last dose must be confirmed against the current ipilimumab product information and your written plan; this guide does not borrow the nivolumab five-month interval. Tell the team promptly if pregnancy is possible.

Sources: nccp

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Sources: nccp

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sources: nccp

Sexual health

Tell the team about sexual symptoms, relationship concerns or practical difficulties with the agreed precautions. Ask for a private discussion and specialist support when needed.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Sources: hodi2010 · nccp

Support alongside treatment

Ask about symptom control, psychological support, finances, transport, work and help at home. Palliative or supportive care can be used alongside anticancer treatment to improve comfort and function.

Sources: nccn-patient

A person you trust

If helpful, bring a family member or supporter and agree what information may be shared. Ask for an interpreter or accessible information when needed; use teach-back to check the schedule and urgent plan.

Sources: nccn-patient

Your document set

Keep consent information, the three-page summary, complete handbook, treatment card, test plan and real contact details together. The QR opens more information but essential instructions remain available in print.

Sources: nccn-patient

TREATMENT REFERENCE · 09

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

Immune toxicity can start or persist after the four-dose course. Keep the alert card, follow-up appointments, steroid-taper or hormone-replacement plan and the individual scan schedule. Report new symptoms after treatment has ended.

Sources: nccp

Restarting or changing treatment

If a visit is missed or held, contact the unit. Do not count a cancelled visit as a dose received. The team reviews whether another dose remains appropriate within the four-dose and 16-week limits; do not assume a delayed dose can always be made up.

Sources: nccp

A treatment summary

Record medicines and doses received, important side effects, admissions, last-dose date, ongoing symptom treatment and the clinician responsible for follow-up. Ask for an updated paper copy for your GP and other treating teams.

Sources: nccn-patient

Your next review

Confirm the next blood test, scan and review, who explains the result, and which symptoms should trigger contact sooner. Keep actual daytime and out-of-hours numbers accessible after treatment stops.

Sources: nccn-patient

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

Median overall survival for ipilimumab alone was 10.1 versus 6.4 months with gp100; HR 0.66, p=0.003. These are group results from that study, not a prediction for you.

Sources: hodi2010 · nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the treatment schedule?

Ipilimumab by IV infusion once every 21 days, up to four doses. Complete within 16 weeks of the first dose, whichever limit comes first.

What if a dose is missed or held?

If a visit is missed or held, contact the unit. Do not count a cancelled visit as a dose received. The team reviews whether another dose remains appropriate within the four-dose and 16-week limits; do not assume a delayed dose can always be made up.

Which symptoms need early contact?

Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.

Which tests are needed?

Blood count, kidney and liver function, glucose and thyroid tests. Review autoimmune disease, previous immune reactions, transplantation, infection, pregnancy possibility and all medicines, including steroids. Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, plus a careful symptom review. Cortisol and other tests are added when symptoms suggest endocrine or other immune effects. Normal routine blood results do not exclude colitis, lung, nerve or other inflammation. Immune toxicity can start or persist after the four-dose course. Keep the alert card, follow-up appointments, steroid-taper or hormone-replacement plan and the individual scan schedule. Report new symptoms after treatment has ended.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

> **Try explaining the plan back:** Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00105 v7. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00105 v7: current regimen source
- NCCN Guidelines for Patients: Melanoma, 2026 — supplied education reference
- Hodi et al., NEJM 2010

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-06.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6724/105_V7_Ipilimumab_.pdf

https://www.nccn.org/patients/guidelines/content/PDF/melanoma-patient.pdf

https://pubmed.ncbi.nlm.nih.gov/20525992/